



EBOLA PRESIDENTIAL FORCE TASK 17

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°098/MVEBDB/08/20/2026

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HIGHLIGHTS (last 24 hours)

As of August 20, 2026, 85 new confirmed cases of Ebola Virus Disease (EVD), including 18 community cases, were recorded in the provinces of Ituri (63 cases), North Kivu (17 cases), and Haut-Uélé (5). No new areas of health is affected.

ACCUMULATIVE CASE 5,375	ACCUMULATIVE DEATH CONFIRMED 2,557	LETHALITY 47.6%	PATIENTS IN ISOLATION/CTE 737	ACCUMULATIVE HEALED 1,167	TRACKING RATES CONTACTS (Of the Day) 85.5%
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Affected Provinces and Affected Health Zones



6 Provinces Affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo & Bas Uélé



56 out of 151 health zones affected (37.1%)



Summary of key points from the last 24 hours

In the past 24 hours, **85 new confirmed cases**, including **18 community deaths** from Ebola Virus Disease (EVD), have been reported in the provinces of **Ituri (63 cases)**, **North Kivu (17)** and **Haut Uélé (5)**.

Over the past 24 hours, **15 patients** have been declared recovered after receiving two negative Ebola virus (EV) test results (9 in Ituri, 4 in Haut-Uélé, and 2 in North Kivu). On the other hand, **23 deaths within Ebola treatment centers** have been recorded, including 19 in Ituri, 2 in Haut-Uélé, 1 in Tshopo, and 1 in North Kivu.

From the start of the epidemic until August 20, 2026, **5,375 confirmed cases**, including **2,557 deaths**, were reported in the DRC, representing a case fatality rate of **47.6 %**. Ituri remains the epicenter of the epidemic, accounting for 83.9% of cumulative confirmed cases and 74.1% of new cases reported in the last 24 hours.

Overall, 56 of the 151 health zones in the six provinces have been affected since the start of the epidemic in the DRC. Ituri has 28 health zones affected out of 36, followed by North Kivu (12/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34) and Bas-Uélé (2/11).

Distribution of confirmed cases and deaths by affected province

(As of August 20, 2026)

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	4,510	2009	44.5%	28/36 (77.8%)	63
North Kivu	680	463	68.1%	12/34 (35.3%)	17
Haut-Uélé	165	75	45.5%	6/13 (46.1%)	5
Tshopo	15	8	53.3%	7/23 (30.4%)	0
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Bas Uélé	2	1	50.0%	2/11 (18.2%)	0
Total	5,375	2,557	47.6%	56/151 (37.1%)	85

Confirmed cases and deaths by province and health zone (situation as of August 20, 2026)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	4510	2009	44.5%	63	6	19	25
Adja	11	1	9.1%				0
Aru	7	6	85.7%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%	1	0		0
Bamboo	126	28	22.2%	12	2		2
Boga	1	1	100.0%				0
Bunia	1254	391	31.2%	10	2		2
Damascus	27	9	33.3%	2	0		0
Drodro	13	6	46.2%				0
Fataki	72	30	41.7%				0
Gethy	5	2	40.0%				0
Kambala	2	0	0.0%				0
Kilo	31	12	38.7%				0
Komanda	54	37	68.5%	1	0		0
Lita	190	106	55.8%	4	0		0
Logo	11	5	45.5%				0
Lolwa	17	4	23.5%				0
Mahagi	4	1	25.0%				0
Mambasa	14	7	50.0%				0
Mandima	25	16	64.0%				0
Mangala	188	105	55.9%	5	0		0
Mongbwalu	595	282	47.4%	2	0		0
Nia-Nia	185	105	56.8%	3	0		0

Nizi	580	244	42.1%	13	0		0
Nyankunde	121	35	28.9%				0
Rimba	10	4	40.0%				0
Rwampara	895	334	37.3%	9	2		2
Tchomia	52	25	48.1%	1	0		0
To ventilate	N / A	206	N / A			19	19
North-Kivu	680	463	68.1%	17	10	1	11
Blessed	107	78	72.9%	3	2		2
Butembo	132	99	75.0%	5	1		1
Goma	1	0	0.0%				0
Kalunguta	13	6	46.2%				0
Katwa	323	217	67.2%	7	6	1	7
Kyondo	15	12	80.0%				0
Lubero	1	1	100.0%				0
Mabalako	2	1	50.0%				0
Masereka	7	4	57.1%				0
Musienene	68	39	57.4%	2	1		1
Oicha	6	4	66.7%				0
Vuhovi	5	2	40.0%				0
High-Uele	165	75	45.5%	5	2	2	4
Boma Mangbetu	24	9	37.5%	1	0		0
Gombari	2	1	50.0%	1	0		0
Isiro	60	27	45.0%	2	1	2	3
Pawa	20	14	70.0%				0
Rungu	1	0	0.0%				0
Wamba	58	24	41.4%	1	1		1
Tshopo	15	8	53.3%	0	0	1	1
Bafwasende	1	0	0.0%				0
Kabondo	3	1	33.3%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	6	4	66.7%			1	1
Mangobo	2	2	100.0%				0
Tshopo	1	0	0.0%				
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	2	1	50.0%	0	0	0	0
Buta	1	1	100.0%				0
Viadana	1	0	0.0%				
Total	5375	2557	47.6%	85	18	23	41

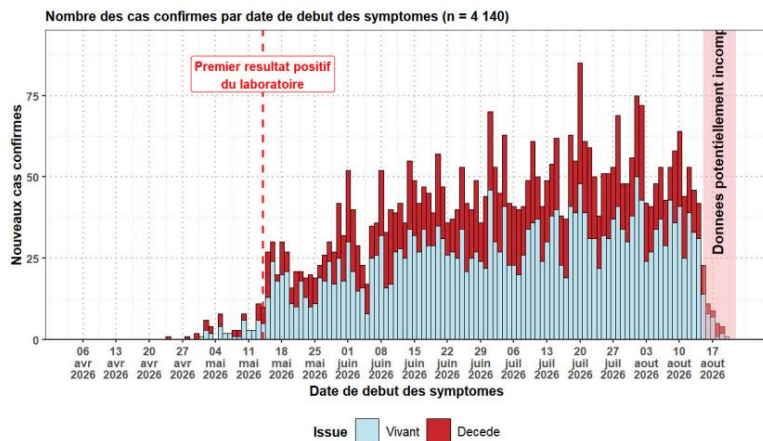
*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

NA: Not Applicable.

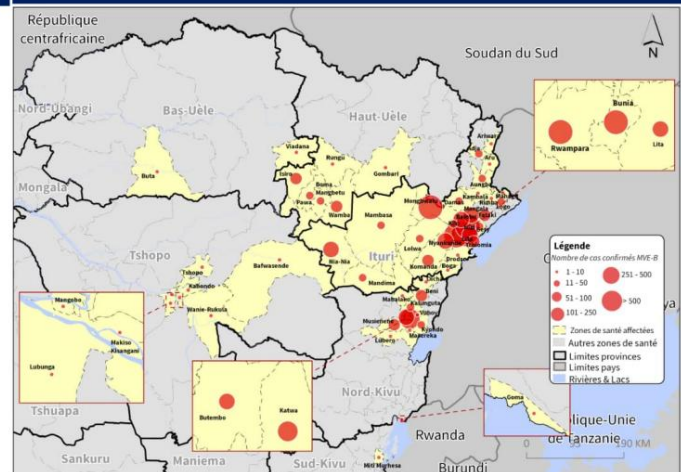
Status of alerts notified by province, as of August 20, 2026

DPS	New Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (as of today)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	20	4	24	13	4	7	0	17	10
Bas Uélé	5	0	5	4	0	1	0	4	0
Ituri	598	65	663	104	43	355	4	147	83
North Kivu	754	56	810	70	46	603	0	56	88
South Kivu	11	0	11	5	0	6	0	5	3
Tshopo	74	1	75	11	1	63	0	10	4
Total General	1462	126	1,588	207	94	1035	4	239	188

Evolution of cases by date of symptom onset over the last 21 days



MAPPING OF CASES over the last 21 days



1. RESPONSE ACTIONS BY PILLAR

1.1. Coordination

1.1.1. Main Actions

- **Haut-Uélé** : Holding of SG meetings; validation of the daily report sent to the Governor of the province and monitoring of response activities in DHIS2.
- **North Kivu** : continued support to the sub-coordinations of Beni, Butembo and Katwa; transmission to WHO of the request for support for the construction of the ITAV and Musienene CTEs; working session with the Kalunguta care committee.
- **Ituri** : continued evaluation of the technical platform of the CTEs using the 5S tool, 14 CTEs having completed their encoding in Kobo Tools; joint PECH-BCZ-OMS visit to Nzere (ZS of Bunia) to assess the feasibility of setting up a transit center.

- **Tshopo** : briefing of 11 provincial inspectors on the management of the Ebola response.
- **Bas-Uélé**: Coordination meeting and monitoring of response activities

1.2. Epidemiological surveillance

1.2.1. Main Actions

At the end of the day on August 20, 2026, 1,588 alerts were recorded, including 1,340 (84.4 % were verified in the six affected provinces. Thus, 301 (22.5%) were validated as suspected cases and 239 (79.4%) were investigated, of which 188 were transferred to CT/CTE.

The proportion of contacts monitored in the last 24 hours has risen to 85.5% (19,674 seen out of 23,001 to be monitored), reaching the 85% threshold: Ituri 89.3% (12,365/13,844), Tshopo 100.0% (488/488) and North Kivu 82.1% (5,929/7,222). Haut-Uélé 61.6% (892/1,447) and Bas-Uélé 59.5% (110/233).

1.2.2. Challenges

Poor contact tracing around confirmed cases mainly in the provinces of Haut Uélé and Bas Uélé due to insufficient response teams.

1.3. Update on checkpoint and point of entry (PoC/PoE) activities

1.3.1. Main actions

- One (1) alert was notified to the PoE/PoC in the last 24 hours, in Ituri (PoC Marabo, AS Marobo, ZS of Nyakunde), validated after investigation and referred to the CTE of Komanda; North Kivu, South Kivu, Tshopo and Haut-Uélé did not notify any alert.

No lifeless bodies or problematic contacts were intercepted at the points of entry.

Monitoring of indicators at PoE/PoC by province, as of August 20, 2026

Indicators	Ituri	North Kivu	South Kivu	Overall
Proportion of PoC enabled and PoE enhanced	100%	100%	100%	100%
Number of people who have switched to PoE/PoC	140,489	78,909	84,432	303,830
% of travelers screened % of	98.0%	98.0%	100%	98.6%
travelers who washed their hands	95.0%	98.0%	100%	97.2%
% of travelers made aware	99.3%	98.0%	100%	99.2%
Number of alerts notified	1	0	0	1
Number of validated live alerts	1	0	0	1
Number of lifeless bodies intercepted today	0	0	0	0
Number of problematic contacts intercepted today	0	0	0	0
% of validated live alerts referred to the CT/CTE	100%	N / A	N / A	100%
% of lifeless bodies swabbed	N / A	N / A	N / A	N / A

1.3.2. Challenges

- Insufficient operationalization of PoE/PoC: six PoCs (Dusu, Apodo, Aungba, PK6 Route Niangara, Gossamu and Magambe) are on strike due to non-payment of service providers; in Ituri, completeness has risen to 100% (60 out of 60 reports) but only 9.09% of strategic PoE/PoCs (1 out of 11) are operating without interruption.

1.4. Laboratory

Main actions

- **Ituri : 63 new positive results** (57 living and 6 deaths) out of 344 new samples received and analyzed (**positivity: 18.3 %**).
- **North Kivu : 17 new positive results** (7 living and 10 dead) out of 117 samples analyzed (**positivity of 14.5 %**).
- **Haut-Uélé: 5 new positive results** (3 living and 2 dead) out of 16 samples analyzed (**positivity of 31.2%**).

1.5. Infection Prevention and Control (IPC/EDS)

1.5.1. Main actions

- **In Ituri**, 17 rings were opened out of 52 expected (32.7%) and 67 places of stay decontaminated out of 82 identified (81.7%); 4 allocations of PCI kits, 13 assessments and 57 ESS support were carried out where 189 providers were briefed; out of 82 death alerts received, 45 bodies were collected by swab, 15 failed and 63 EDS carried out.
- **In North Kivu**, 6 rings were opened, 11 FOSA and 2 households decontaminated and 6 ESS assessed; 121 FOSA were supported and 372 providers briefed, with completion reaching 7 out of 12 health zones. Out of 44 death alerts, 44 bodies were collected by swab (100%).
- **In Bas-Uélé**, 10 out of 13 ESS were accompanied and one ring opened in Buta and one in Viadana.
- **In Tshopo**, the premises of the La Famille Hospital Centre (6th transversal, Kabondo Health Zone), the place where the 15th confirmed case passed through, have been decontaminated and the affected household supported (1 out of 1 case, i.e. 100%).
- **In Haut-Uélé**, 4 PCI kits were provided, 6 ESS evaluated, 11 ESS supported and 121 providers briefed in the ZS of Isiro, Wamba, Boma Mangbetu, Pawa, Gombari and Rungu; out of 5 death alerts received, 5 bodies were swabbed and 3 EDS carried out.

1.5.2. Challenges

- Weak implementation of IPC standards: limited ring opening in Ituri (17 out of 52) due to insufficient decontamination teams, unreachable addresses and insecurity in some villages, with 15 swab failures; resistance to EDS in Ituri (Bunia, Nizi, Mongbwalu, Fataki and Damas) and refusal of a family in Rwampara for household decontamination; in Haut-Uélé; in North Kivu, average IPC score of 47.9% in Beni and five health zones (Vuhovi, Kalunguta, Masereka, Lubero and Goma) did not report.

1.6. Continuity of care

1.6.1. Main actions

- **In Ituri**, 89 new admissions and 3 readmissions were recorded, with 116 discharges, including 9 recoveries. Thus, 536 patients are hospitalized in 965 beds, representing an overall occupancy rate of 55.2%.
- **In North Kivu**, 41 admissions were recorded (4 confirmed and 37 suspected) with 60 discharges, including 2 recoveries at the Kitatumba Ebola Treatment Center. Thus, 169 patients are hospitalized out of 206 beds, representing an occupancy rate of 82.0%.
- **In Tshopo**, three new admissions of suspected cases at the CTE HC Kisangani bring the number of isolated patients to 6 (3 confirmed and 3 suspected) for 12 beds, i.e. 50.0% occupancy; a death of a confirmed case occurred on the 9th day of hospitalization.
- **In Bas-Uélé**, 1 patient is in isolation at the Limba transit center (Viadana health zone) for 3 beds, i.e. 33.3% occupancy; no ambulance is available for the response.
- **In South Kivu**, 25 suspected cases remain hospitalized.

1.6.2. Challenges

Low reception capacity and insufficient technical platform of several CTE/CT: saturation of the CTEs of Nizi, Kilo, Logo and Kigonze (suspected) in Ituri, where 43 patients (Kilo 13, Sota 16, Lolwa 9, Mandima 4 and Mangala 1) do not receive any food support and continuous escape of patients in some CTEs of the ZS; slowdown of the work of extension of the CTEs of Nizi, ISTM and Bambu; in North Kivu, several suspected cases from Butembo, Masereka, Vuhovi and Kalunguta are isolated in ESS which are not transit centers; in Tshopo, low capacity of the CTE, power cuts, lack of safe blood and insufficient biological technical platform; in Haut-Uélé, stock shortage of some MEGs at the CTE of the HGR of Isiro, in Bas-Uélé, no ambulance is available for the response.

1.7. Risk communication and community engagement

1.7.1. Main actions

- **In Ituri**, 678 new contacts were identified around the cases. In Haut-Uélé, 216 people were reached during 44 home visits, 719 during 13 educational talks and 312 during mass awareness campaigns around the bereaved family of AS Mayogo (Isiro Health Zone); 5 local radio stations broadcast the messages and 53 IEC tools were distributed.
- **In Ituri**, 66 affected households received support and 78 children were separated or orphaned community-based follow-up.
- **In Tshopo**, 1,519 people were reached by home visits and research active in Bondeko, Mangobo, Tshopo and Lubunga.
- **In North Kivu**, briefing of 34 journalists from Beni on their role in community engagement and awareness-raising by video forum of 100 young motorcyclists and members of the Véranda Mutsanga movement; community dialogue with 78 leaders of the Vutsundo and Vusenzero AS (Butembo Health Zone) and mass communication with 119 young people from the Magheria AS (Masereka Health Zone); 7 cases were covered and 26 household heads affected, and 7 rumors were clarified.

1.7.2. Challenges

Persistent community challenges: in Haut-Uélé, a body was taken from the CTE of the HGR in Isiro and displayed in the city for funeral ceremonies, a family in Boma Mangbetu also refusing hospitalization on the grounds of poor care conditions; persistent rumors in North Kivu (the body bag "does not open the passage to heaven" in Kyondo, the measles-rubella-polio vaccine perceived as a cure against Ebola in Beni) and in Tshopo "virus made by the White man"; demotivation of CREC actors not paid for three months in North Kivu and Tshopo.

1.8. Logistics

1.8.1. Main actions

- In Tshopo, 10 motorcycles and IPC supplies were received, as well as 1,100 kg of medical care supplies from Bunia (basic malaria kits and glucose serum) which are being unpacked; construction work on the isolation units in Madula and Kabondo and the large capacity CTE continues.
- In North Kivu, IPC supplies were delivered to the health zones of Musienene and Beni, and nutritional support supplies (Plumpy Nut) were delivered to the Beni health zone; four vehicles were deployed (Musienene, Beni, Mukulya and Kangote checkpoints, and Kyondo), and the warehouse was renovated with logistical support from WFP. Construction of the triage facility at Vuhovi General Referral Hospital (ULB Cooperation) and rehabilitation of the treatment center at Musienene General Referral Hospital (MSF France) are ongoing.
- In Bas-Uélé, 13 Starlink kits donated by the WHO were received for the 13 health zones, basins were purchased for hand washing and preparation of the chlorinated solution, and the Buta health zone received a second supply of IPC inputs; the transport of samples from the health zones to the Radi-One site is ensured.

1.8.2. Challenges

Persistent logistical needs to support the expansion of the response: completion of the extensions of the CTEs in Nizi, ISTM and Bambu in Ituri and needs for 4 glove boxes (ZS of Fataki, Bambu-mine, Lita and Rwampara), cryotubes and cryoboxes; three ambulances are also out of service (1 in Bambu, 1 in Bunia and 1 in Nizi); deployment of IPC inputs to the 6 zones in the west and north of Bas-Uélé, where a pre-shortage has been reported and no medicine is pre-positioned for supportive treatment; in Tshopo, mobility deficit (16.0% of motorcycles available, i.e. a gap of 168); in Haut-Uélé, lack of a vehicle dedicated to the EDS teams and mobility for the IPC and CREC pillars.

1.9. Security

1.9.1. Main actions

In North Kivu, the pillar ensured the control and supervision of security agents assigned to the Points of Execution (PoE) and Points of Control (PoC) in Beni, provided security support to an Emergency Response Team (EDS) at the Bundji cemetery and to two decontamination teams in the Butsili health zone (AS), and updated the mapping of recommended accommodation sites for Beni and Butembo. In Bas-Uélé, a joint operation with the provincial ministers of the interior and health helped overcome resistance in Bambole (Buta health zone).

1.9.2. Challenges

In Haut-Uélé, two Red Cross vehicles accompanying a dead body were vandalized 10 km from Isiro and the volunteers were harassed; an ambulance from the Pawa Health Zone was attacked during the transfer of a suspect; a joint surveillance-CREC team was threatened on Senze Avenue (Bazanga district, Isiro city) during the investigation of an alert.

1.10. PSEA (Prevention of Sexual Exploitation and Abuse)

1.10.1. Main actions

In Tshopo: Training of community, military and police leaders on PSEA and MVE was supported by the communication pillar.

1.10.2. Challenges

Coverage of PSEA activities is still insufficient, limited to 4 out of 36 health zones in Ituri, lack of mapping of stakeholders, insufficient mass awareness materials and lack of transport and computer equipment for the pillar; Coverage remains limited to 4 of the 36 health zones of Ituri (11.1%) and only two partners (WHO and Africa CDC) report their PSEA activities.

KEY MESSAGES

- § Transmission of Ebola virus disease remains active, with 85 new cases including 18 community deaths and 23 intra-CTE recorded in the last 24 hours, bringing the national total to 5,375 confirmed cases and 2,557 deaths.
- § Ituri remains the epicenter of the epidemic, concentrating 83.9% of cumulative confirmed cases and 74.1% of new cases reported in the last 24 hours and a greater expansion (28 of the 36 health zones affected).
- § Surveillance shows 79.4% of suspected cases investigated while the follow-up rate (85.5%) corresponds to the threshold of 85.0%.
- § Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains.

TIMELINE OF KEY EVENTS

- § From April 24 to May 15, 2026, the epidemic passed from the detection of the first suspected cases to the Biological confirmation of the Ebola Bundibugyo virus leading to the official declaration of the 17th Ebola Virus Disease Outbreak in the Republic Democratic Republic of Congo.
- § Between May 17 and 18, 2026, the authorities national authorities, with the support of partners, have strengthened the response by declaring a state of emergency public health of national scope then continental, accompanied by the mobilization of 64 and support.



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